the different types of Inflammation

Acute

Redness
Heat
Swelling
Obvious & immediate reactions

↓

Resolution

Chronic

Sympoms not always obvious
Persistent pain (not "sharp", rather are dull)
Fatigue
Headaches
Gut distrubances
Brain Fog

↓

If not managed:
Chronic disease development
Autoimmune disease



Clinical Features

The 5 ancient cardinal signs of inflammation are



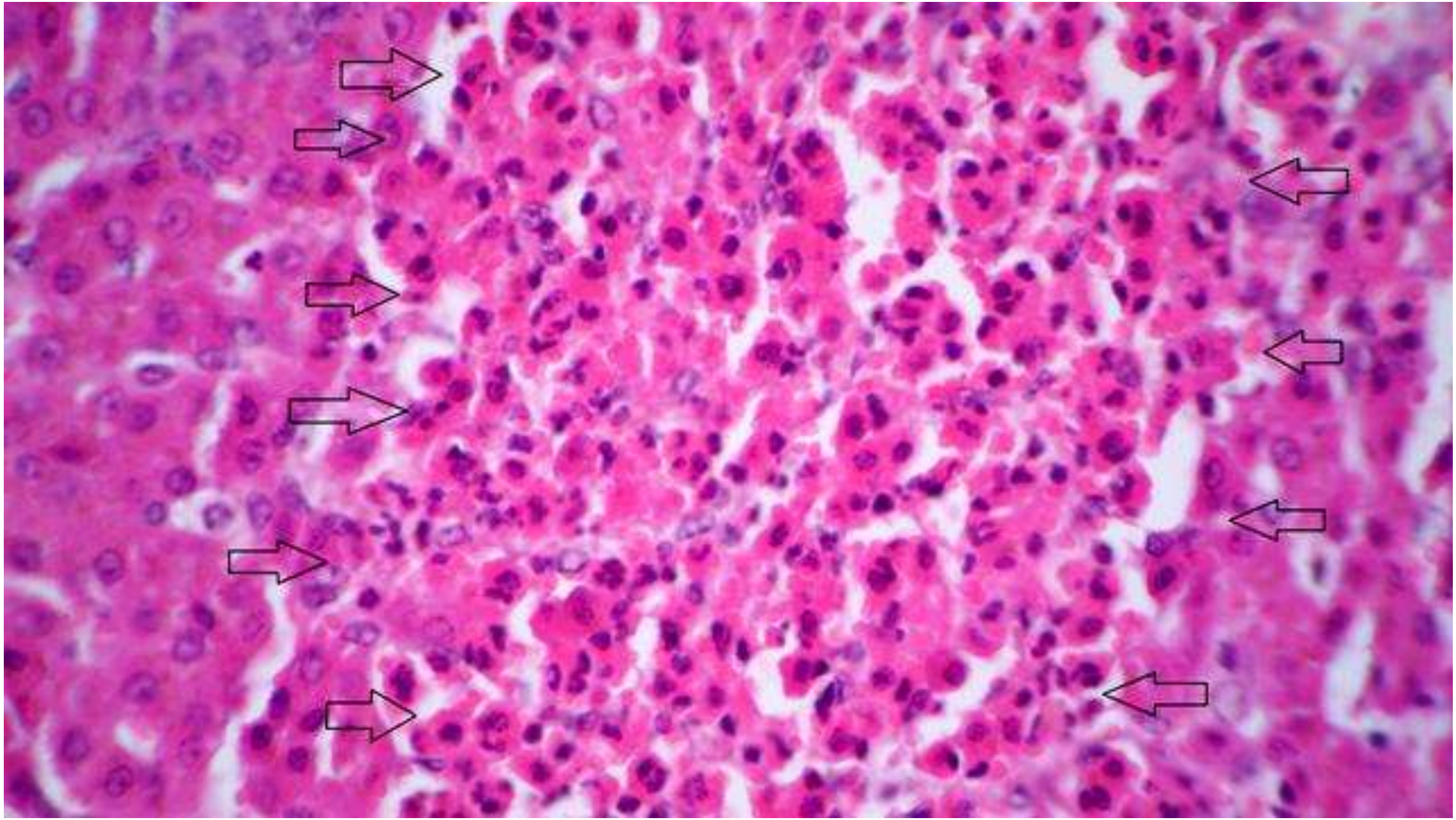
Tumor:-swelling

Rubor :– redness

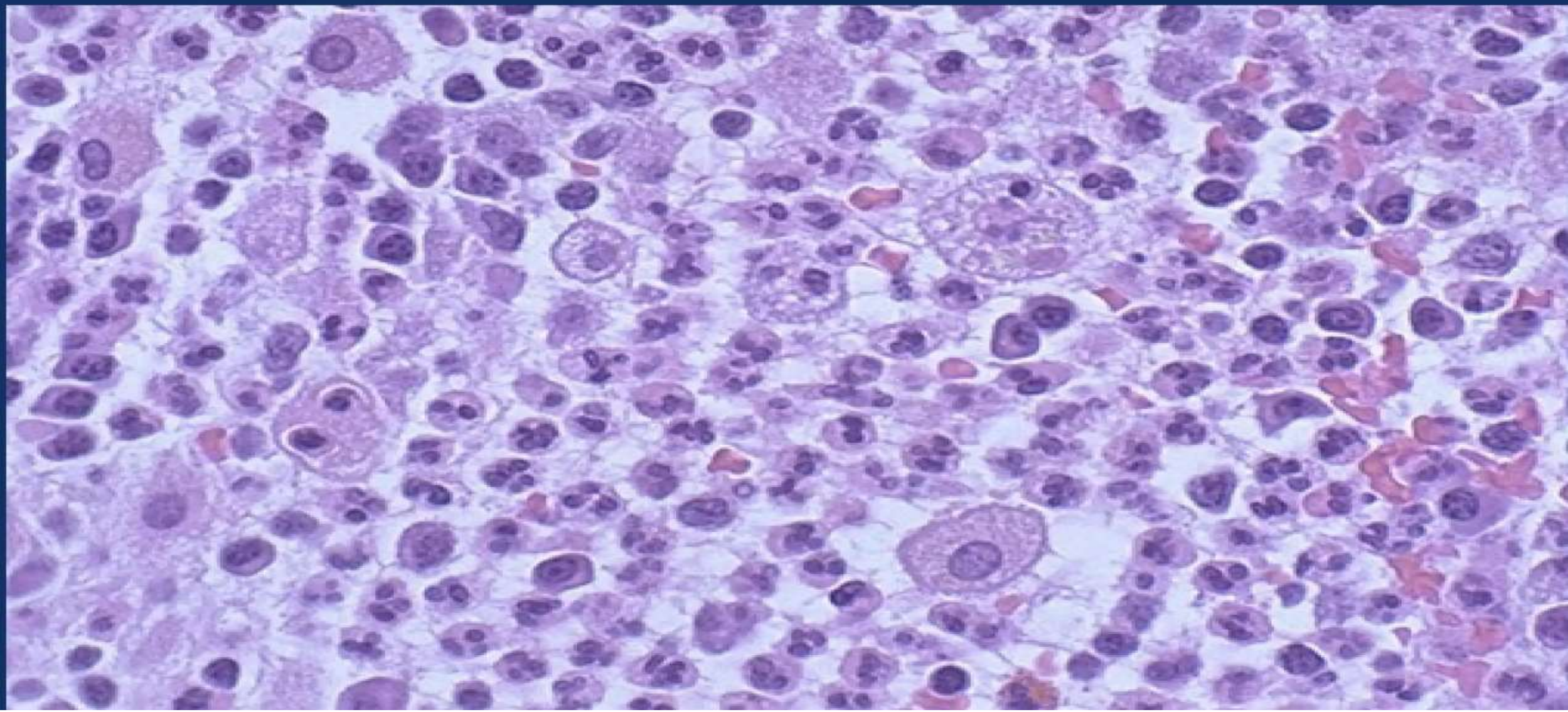
Calor: – warmth

Dolor :– pain

Functio Laesa :–
loss of function



INFLAMMATORY CELLS

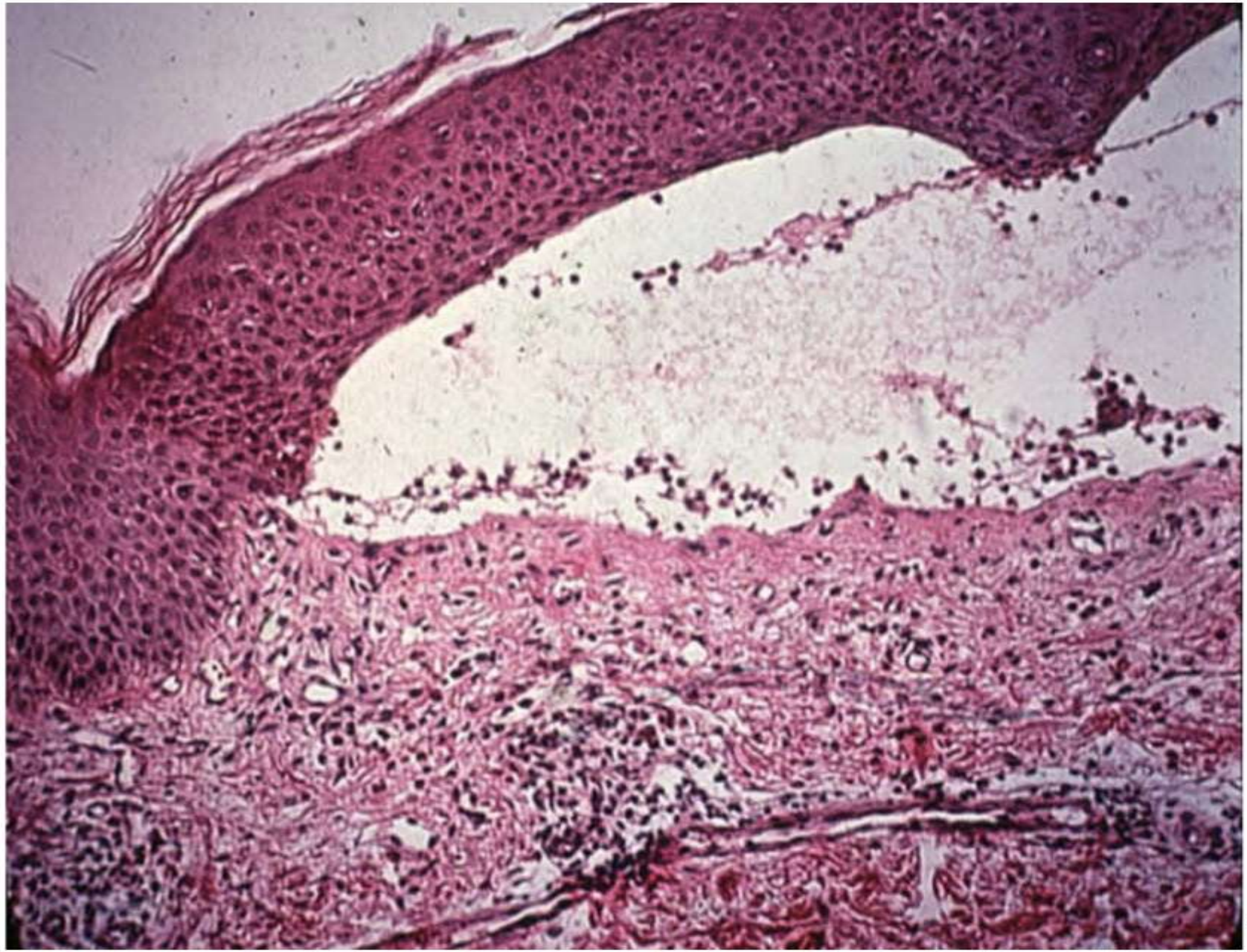


- What are the morphologic pattern of acute inflammation



Serous inflammation

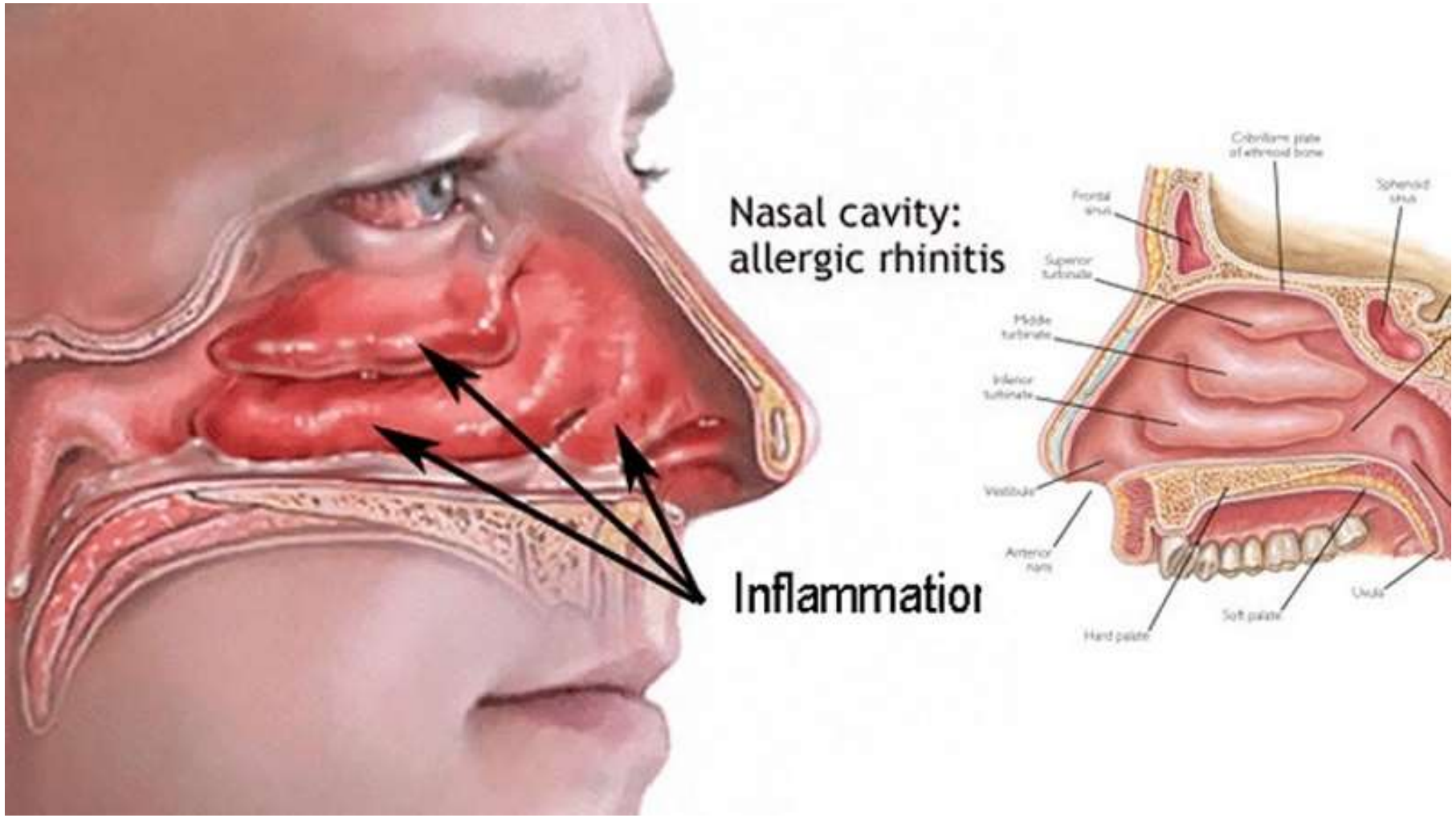




< Back

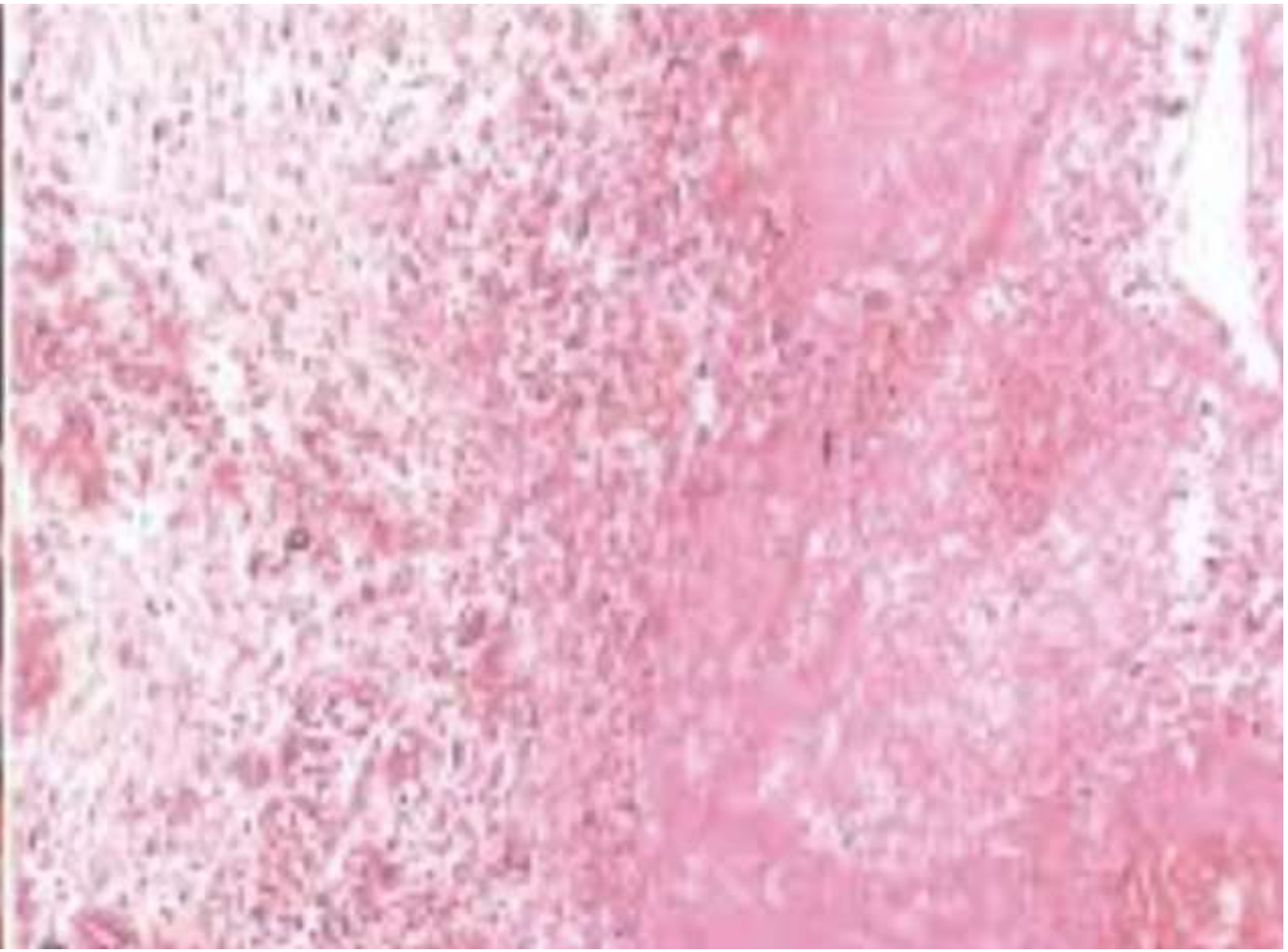
Serous inflammation: Microscopic medium power displays the disruption of epidermis at the basal level leaving a clear-fluid-filled bullous lesion.

What is the diagnosis?

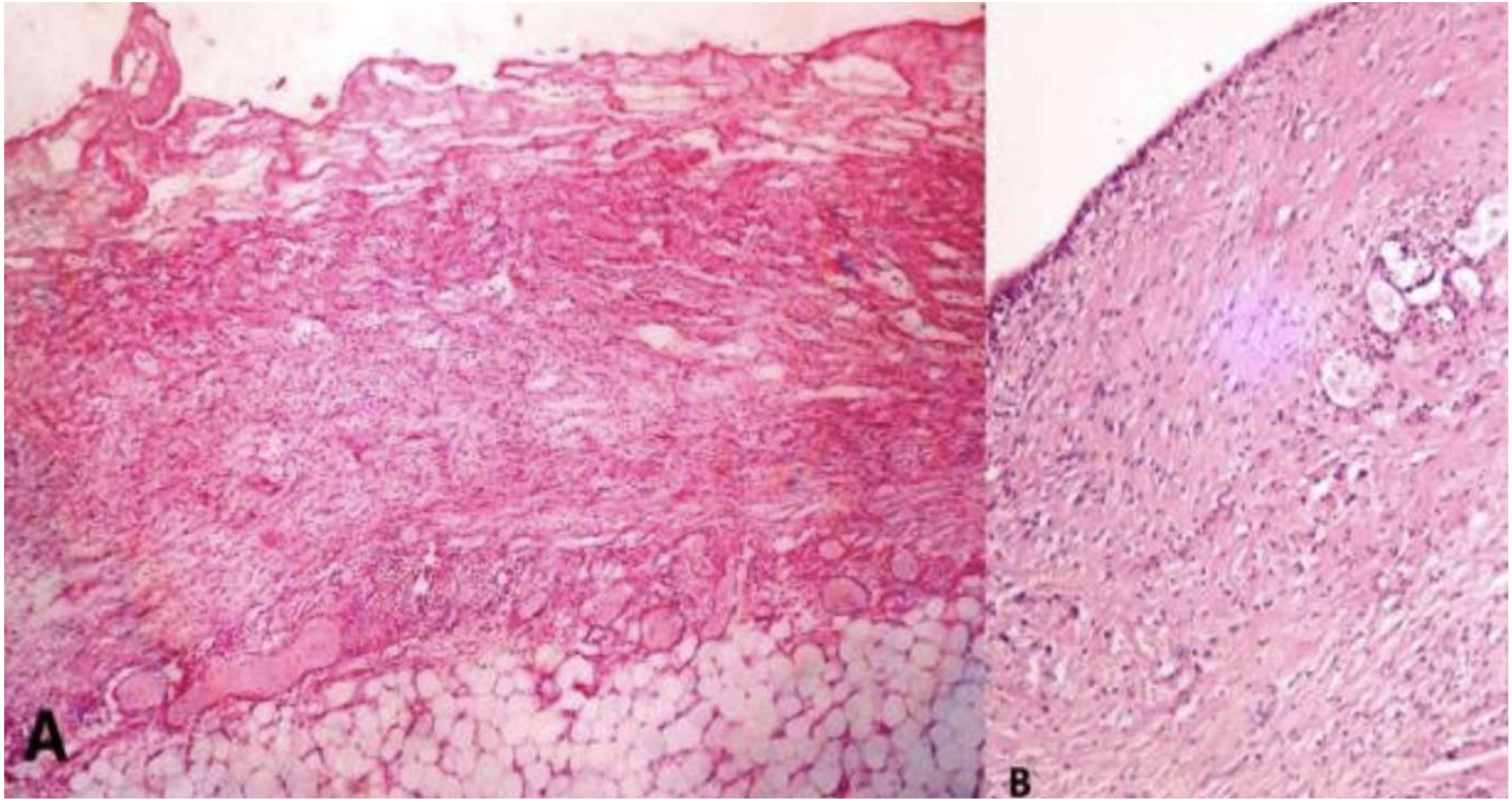


Fibrinous inflammation

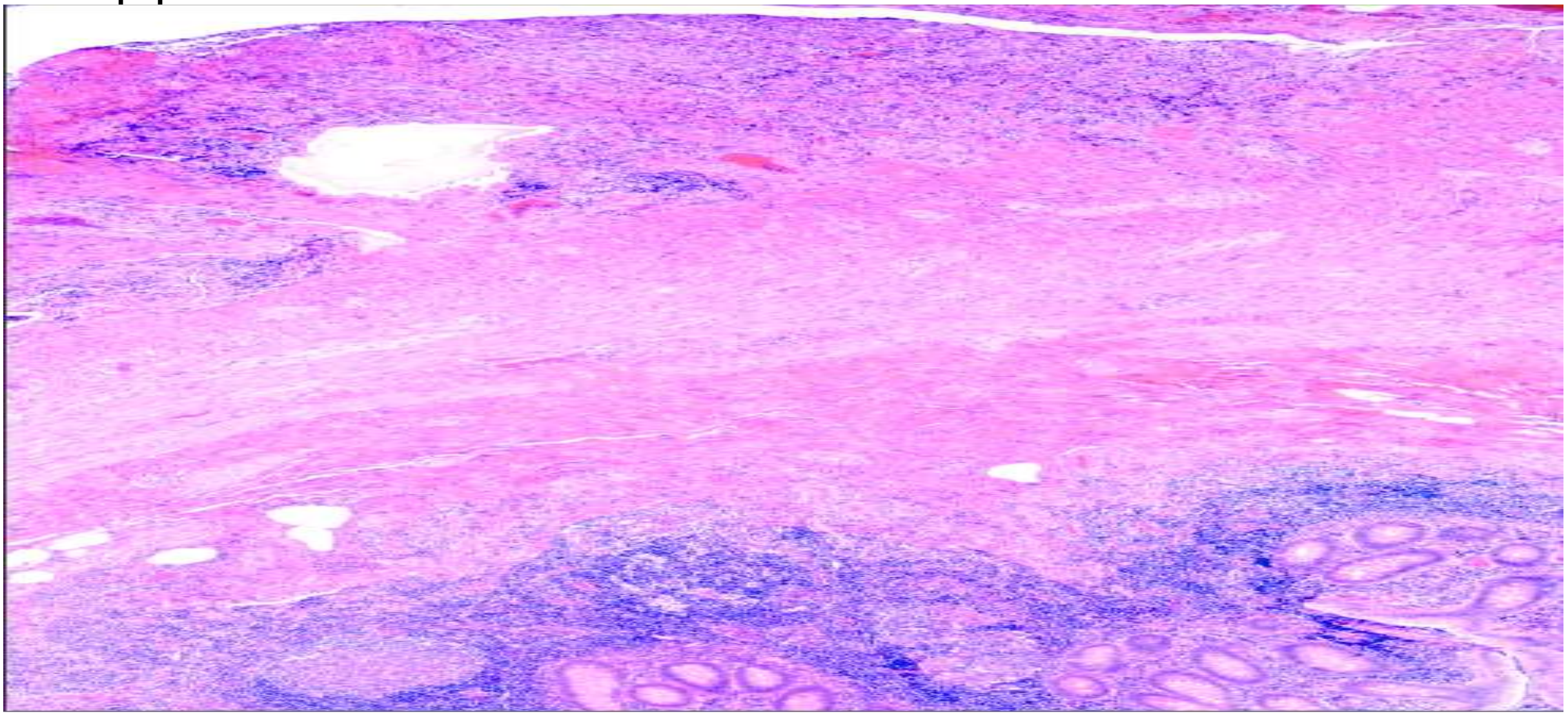




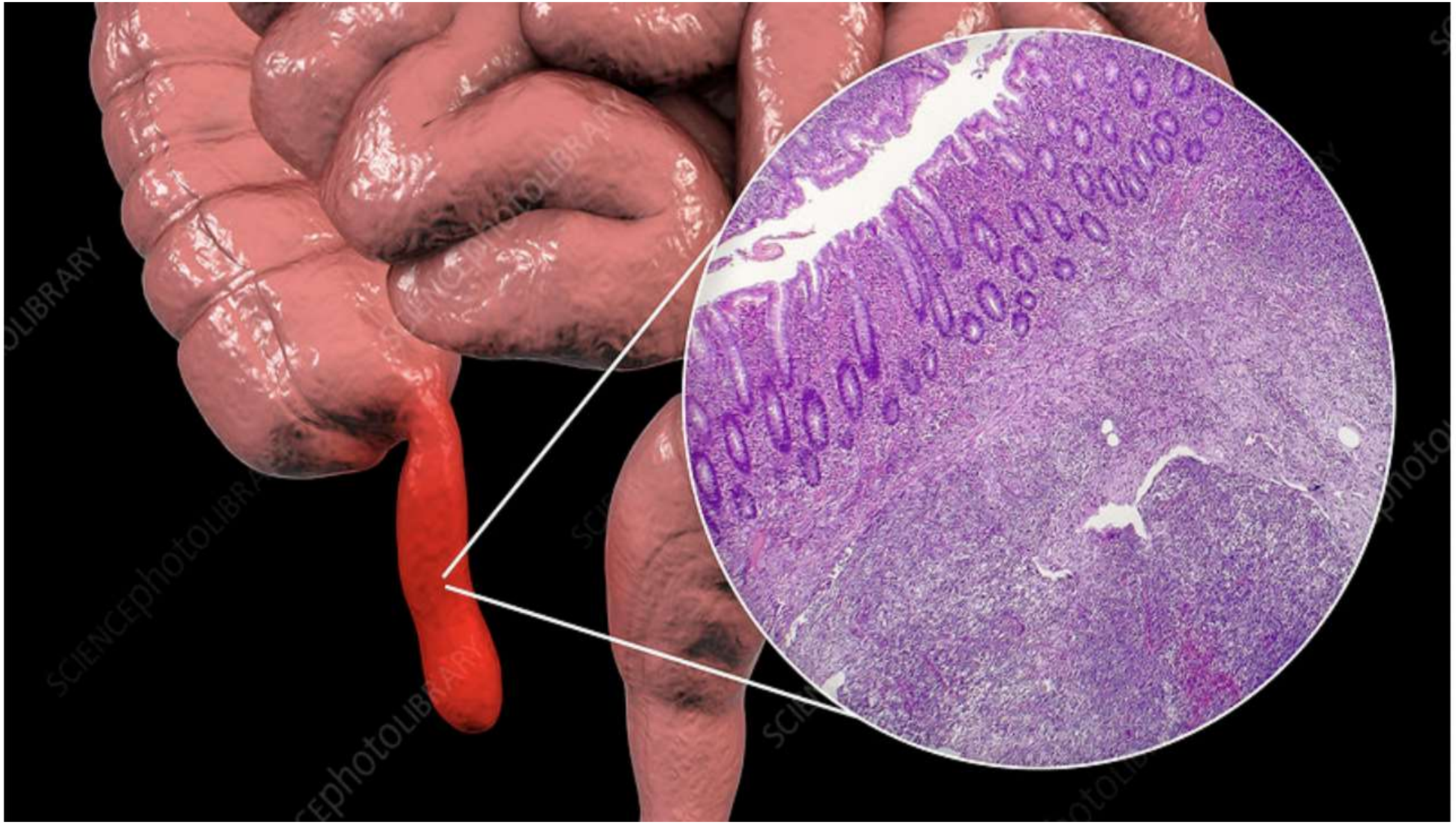
Pericardium showing extensive fibrinous exudate with inflammatory cell infiltrates suggesting fibrinous pericarditis

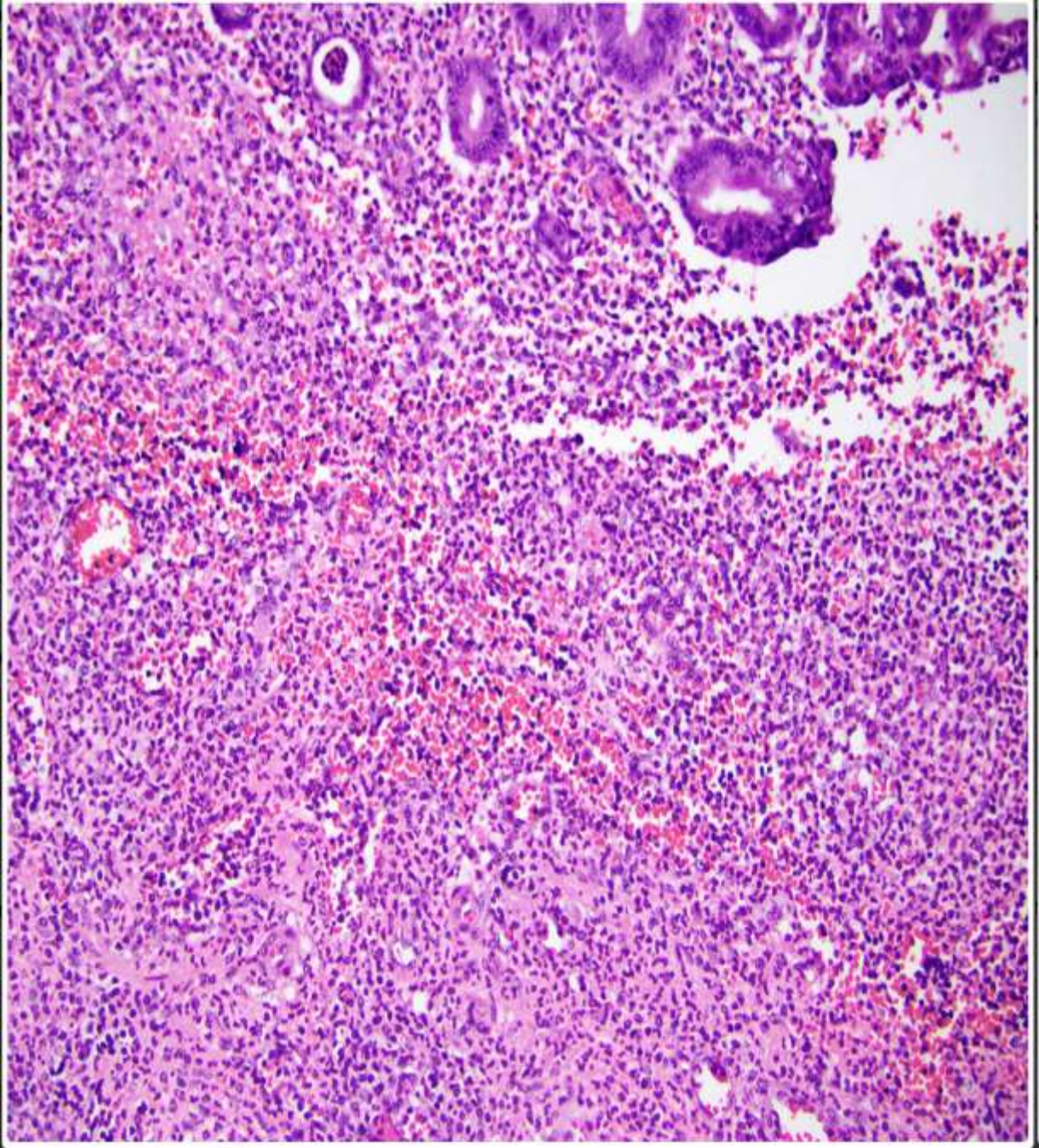


Fibrinous inflammation
Appendix

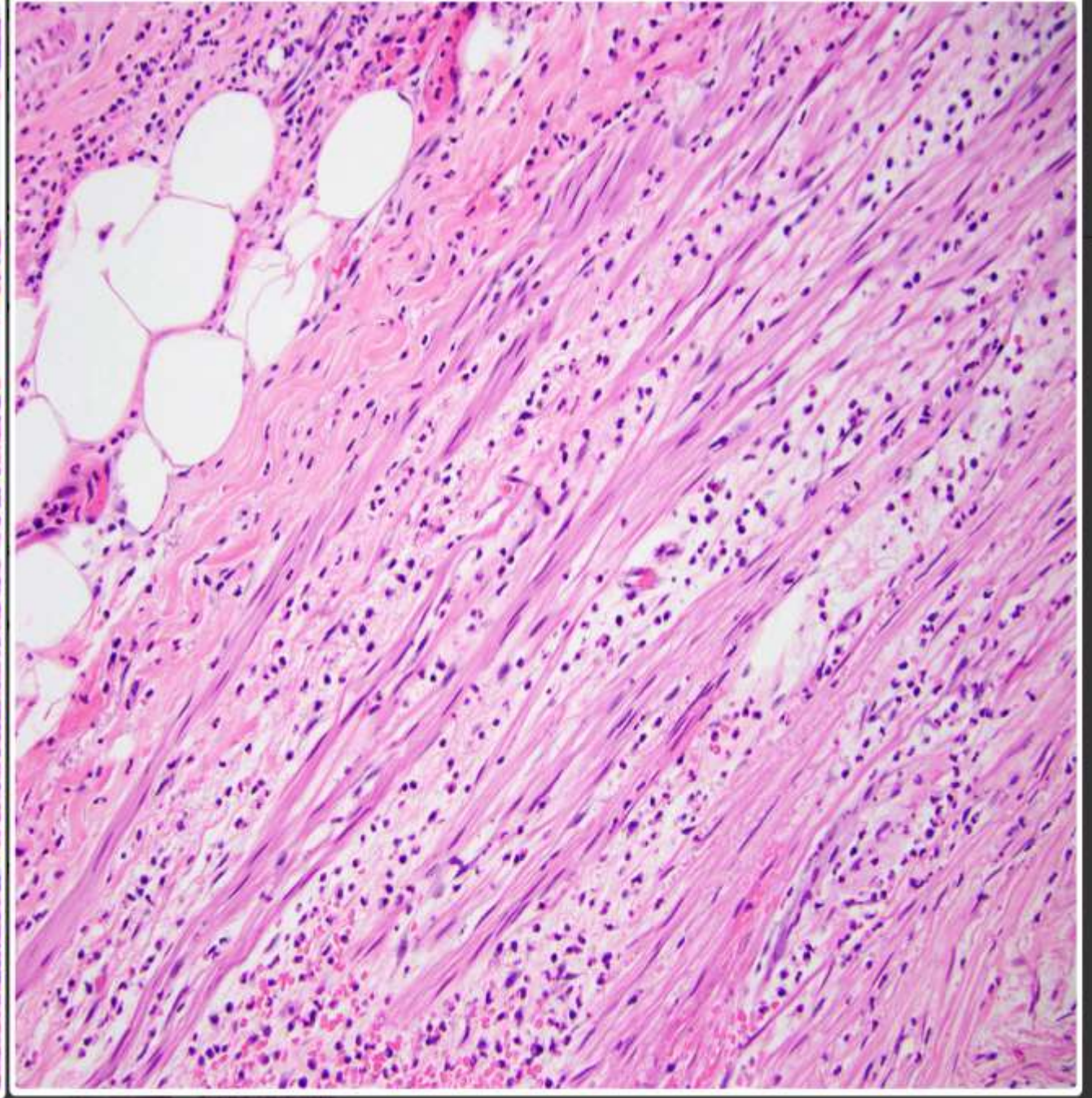


Suppurative inflammation



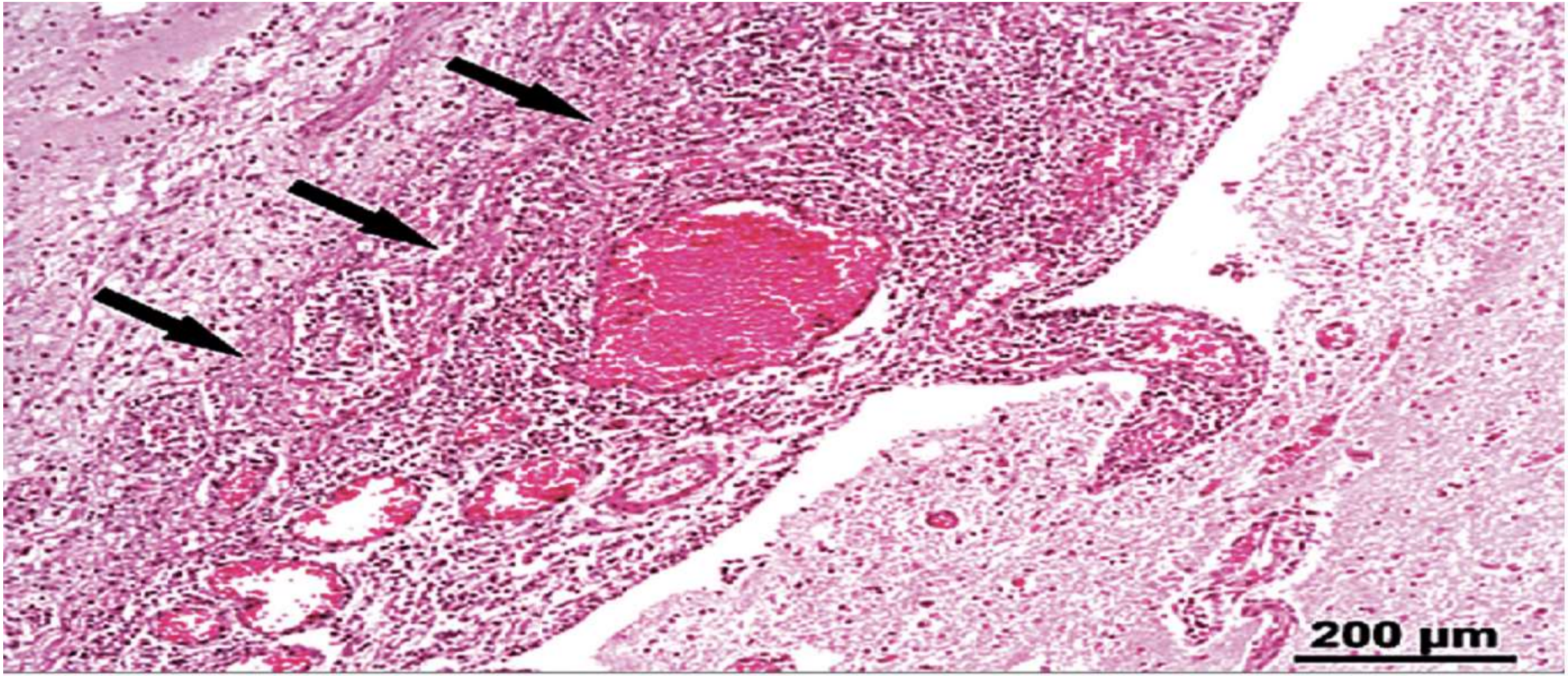


Marked neutrophilic infiltration of appendiceal wall.

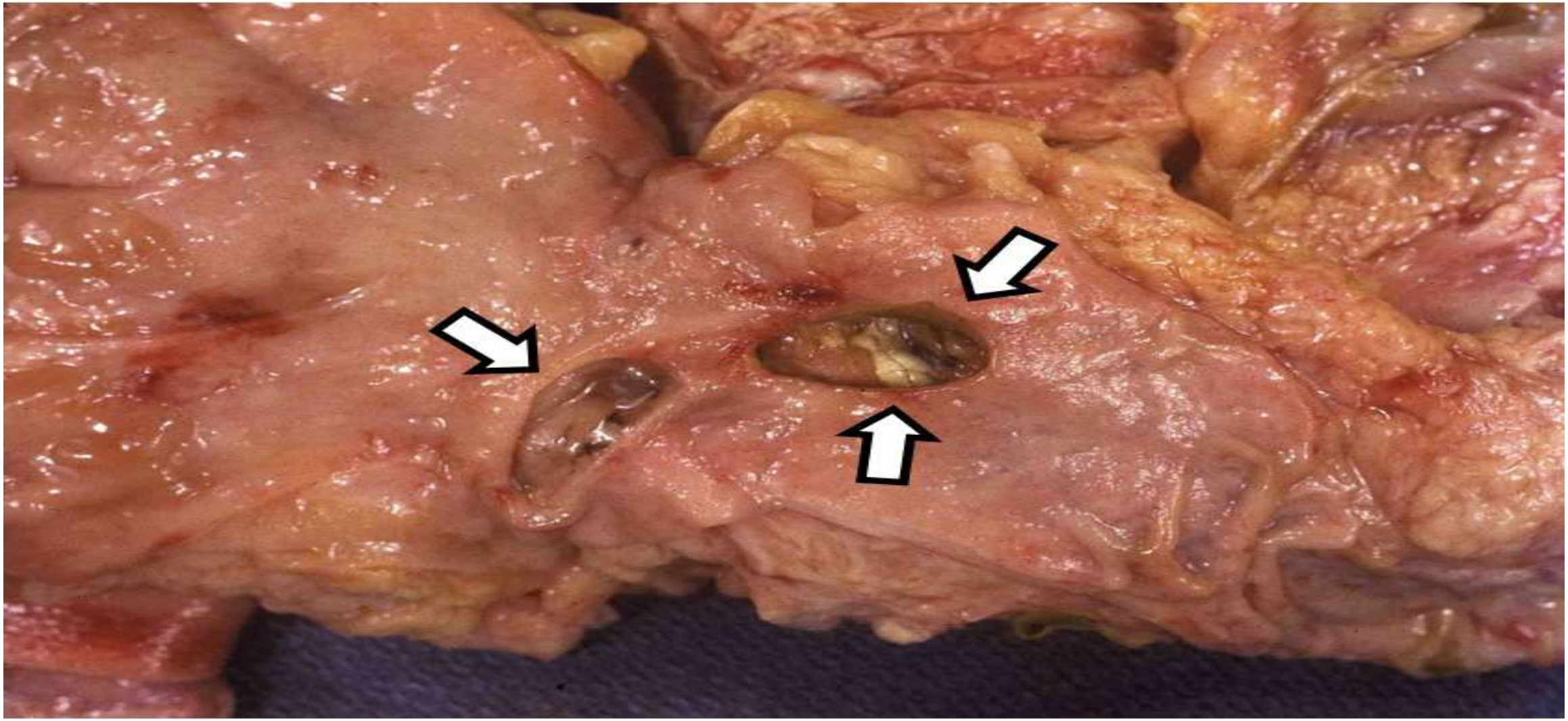


Marked neutrophilic infiltration of appendiceal wall.

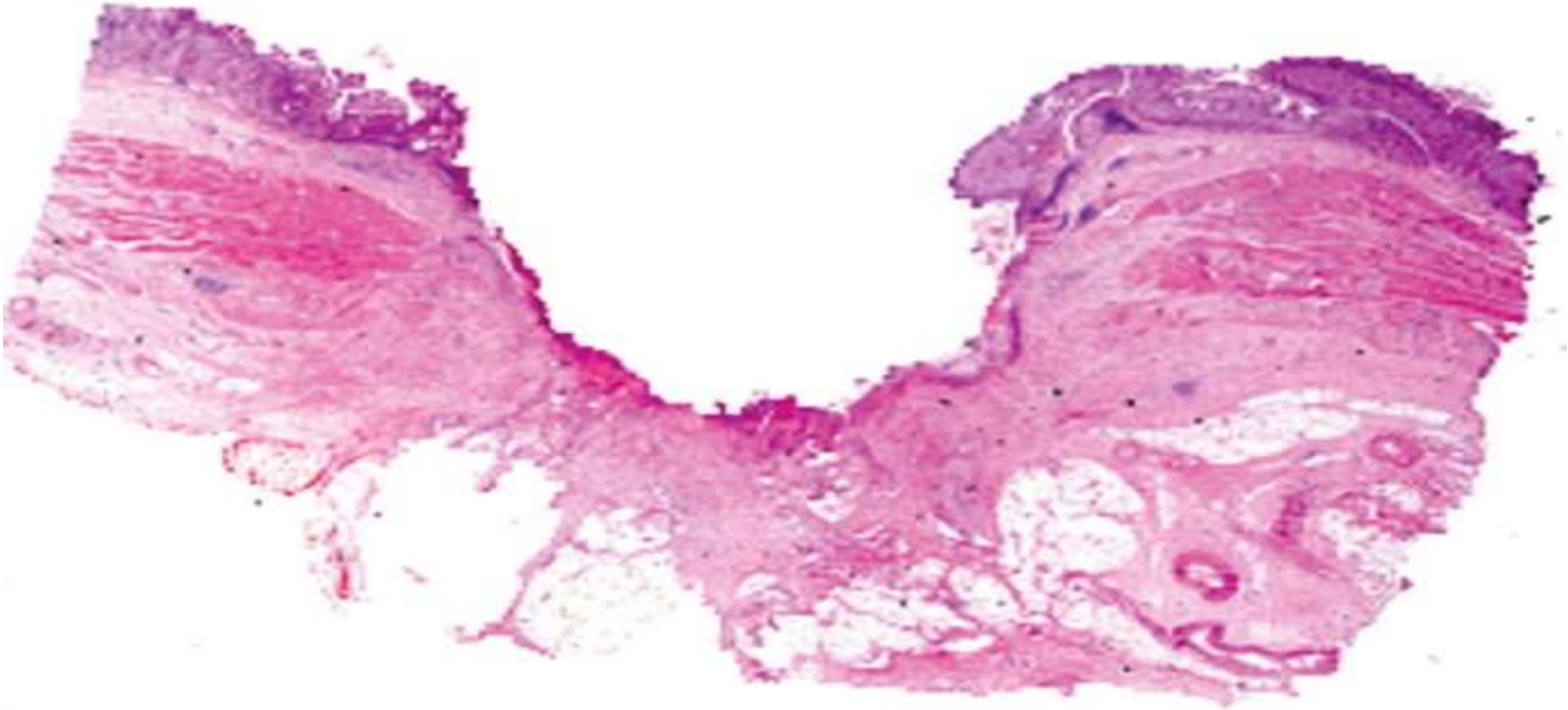
Brain abscess



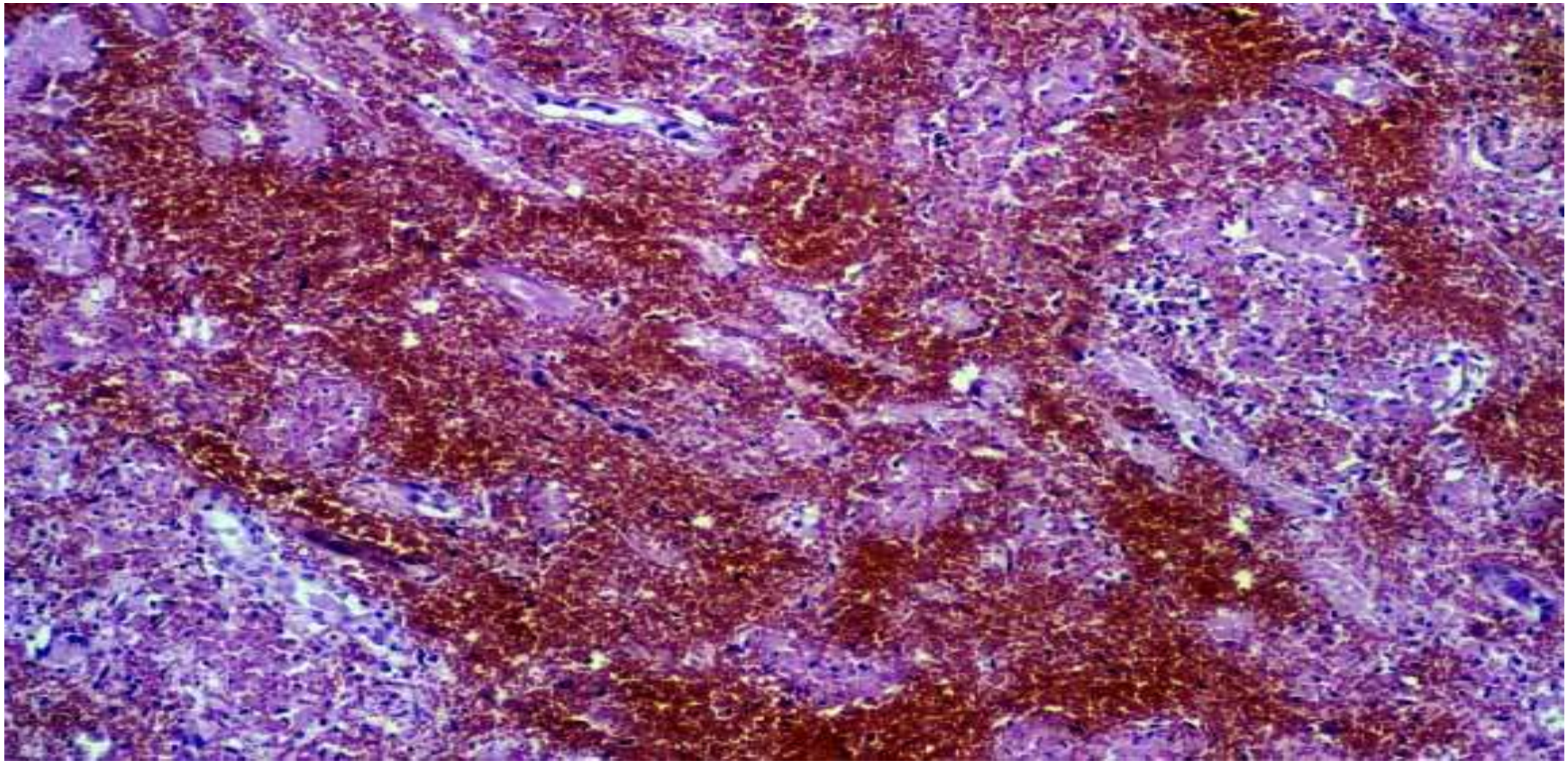
Gross/ peptic ulcer



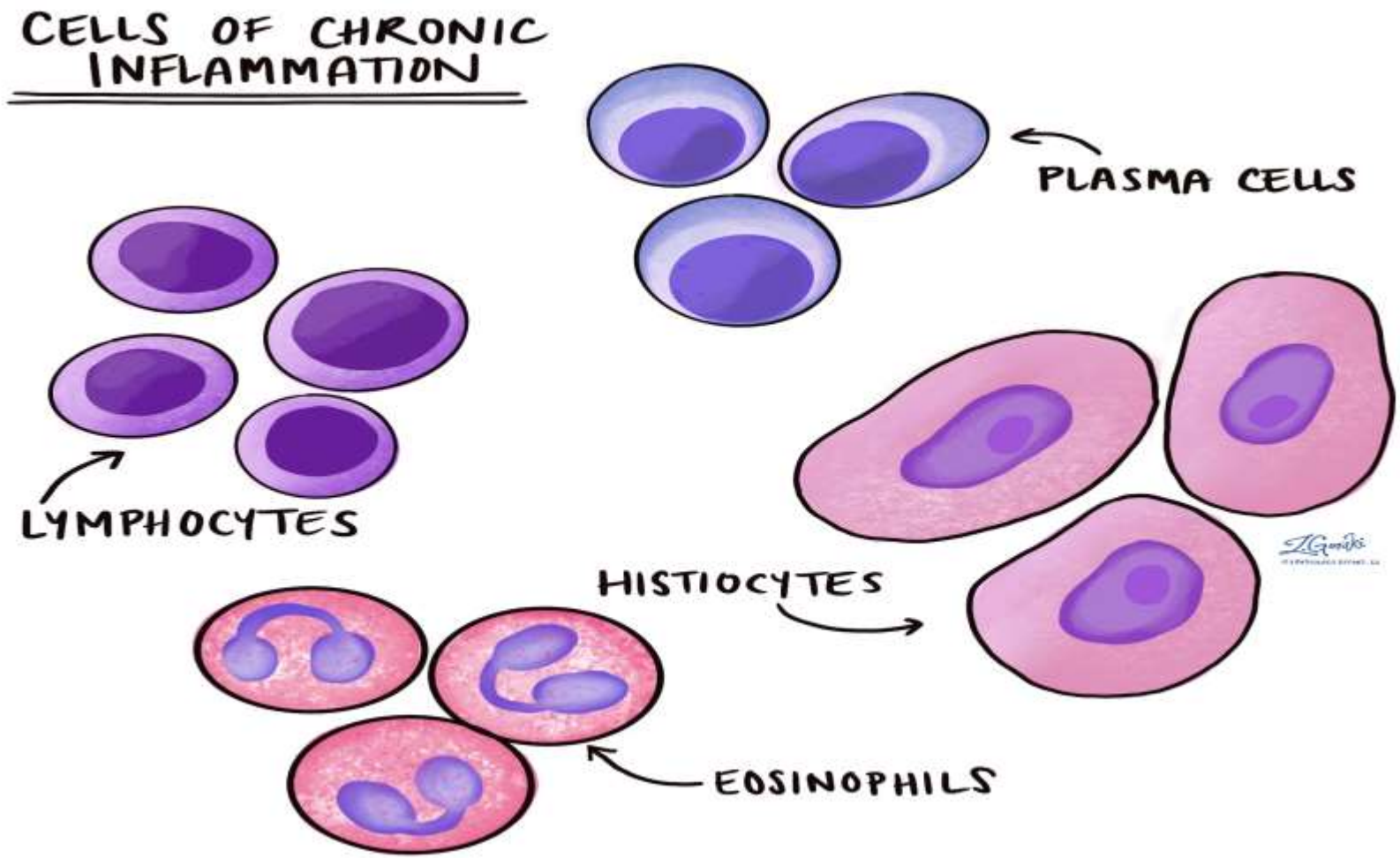
Peptic ulcer



What is the diagnosis?



Chronic inflammatory cells



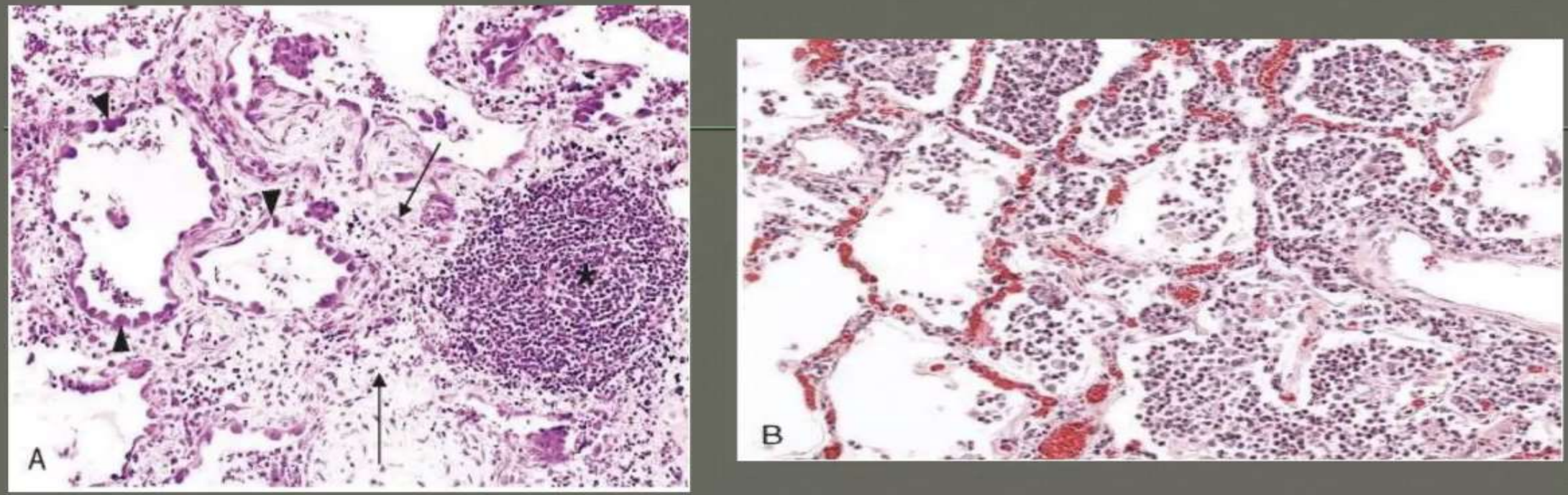
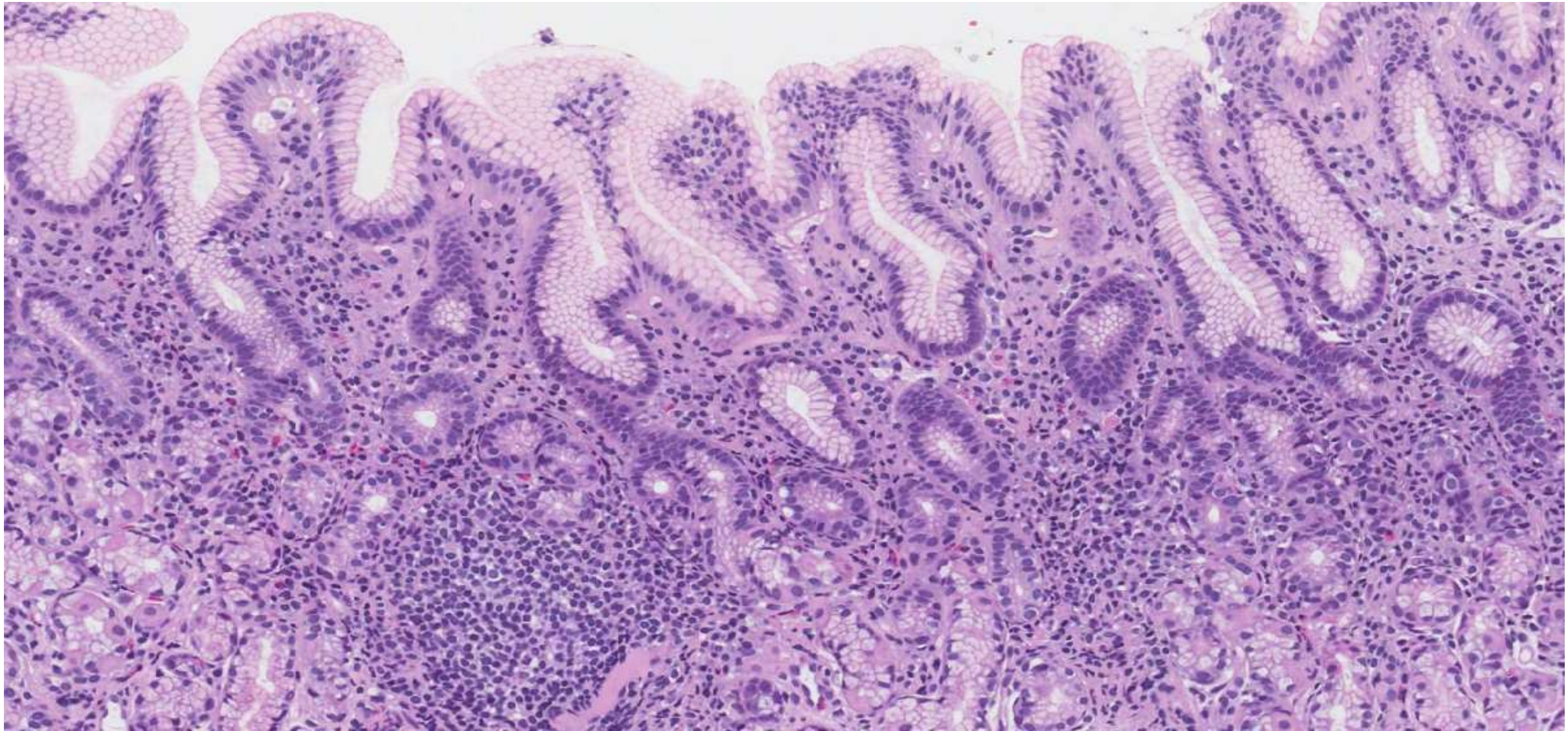
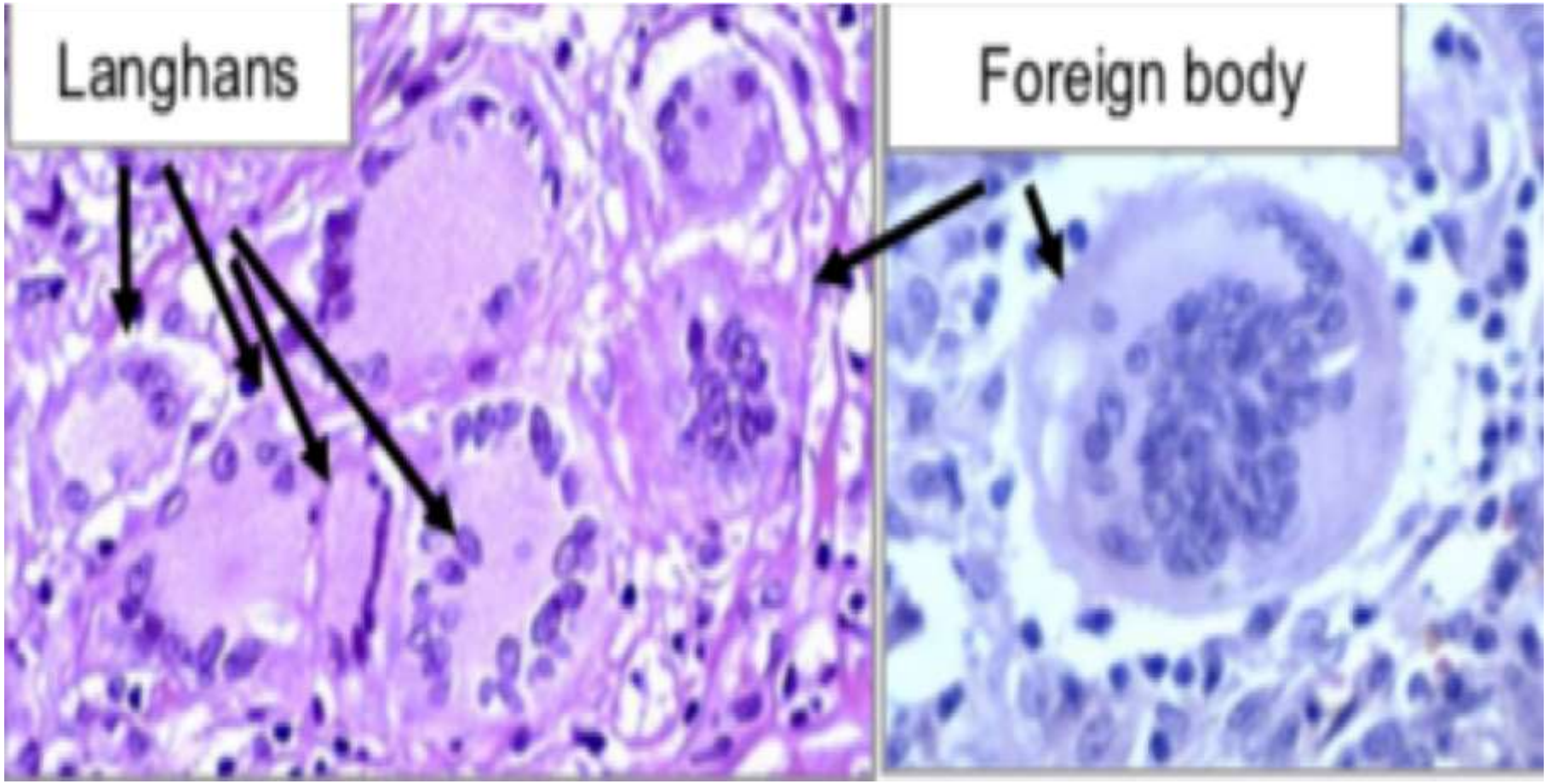


FIGURE 2-22 **A**, Chronic inflammation in the lung, showing all three characteristic histologic features: (1) collection of chronic inflammatory cells (*), (2) destruction of parenchyma (normal alveoli are replaced by spaces lined by cuboidal epithelium, *arrowheads*), and (3) replacement by connective tissue (fibrosis, *arrows*). **B**, By contrast, in acute inflammation of the lung (acute bronchopneumonia), neutrophils fill the alveolar spaces and blood vessels are congested.

M/F Chronic inflammation

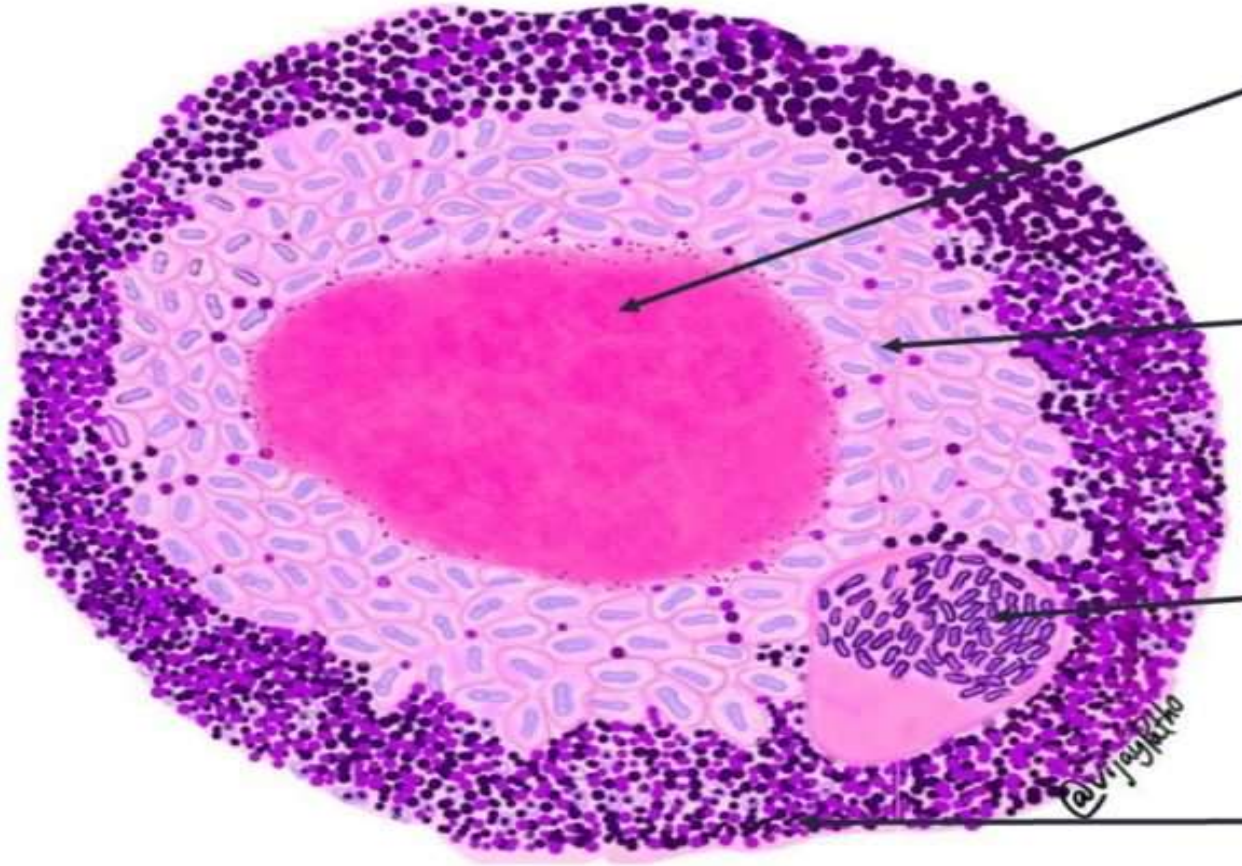


Giant cell



Granuloma

TUBERCULOUS LYMPHADENITIS: **Necrotizing granuloma**



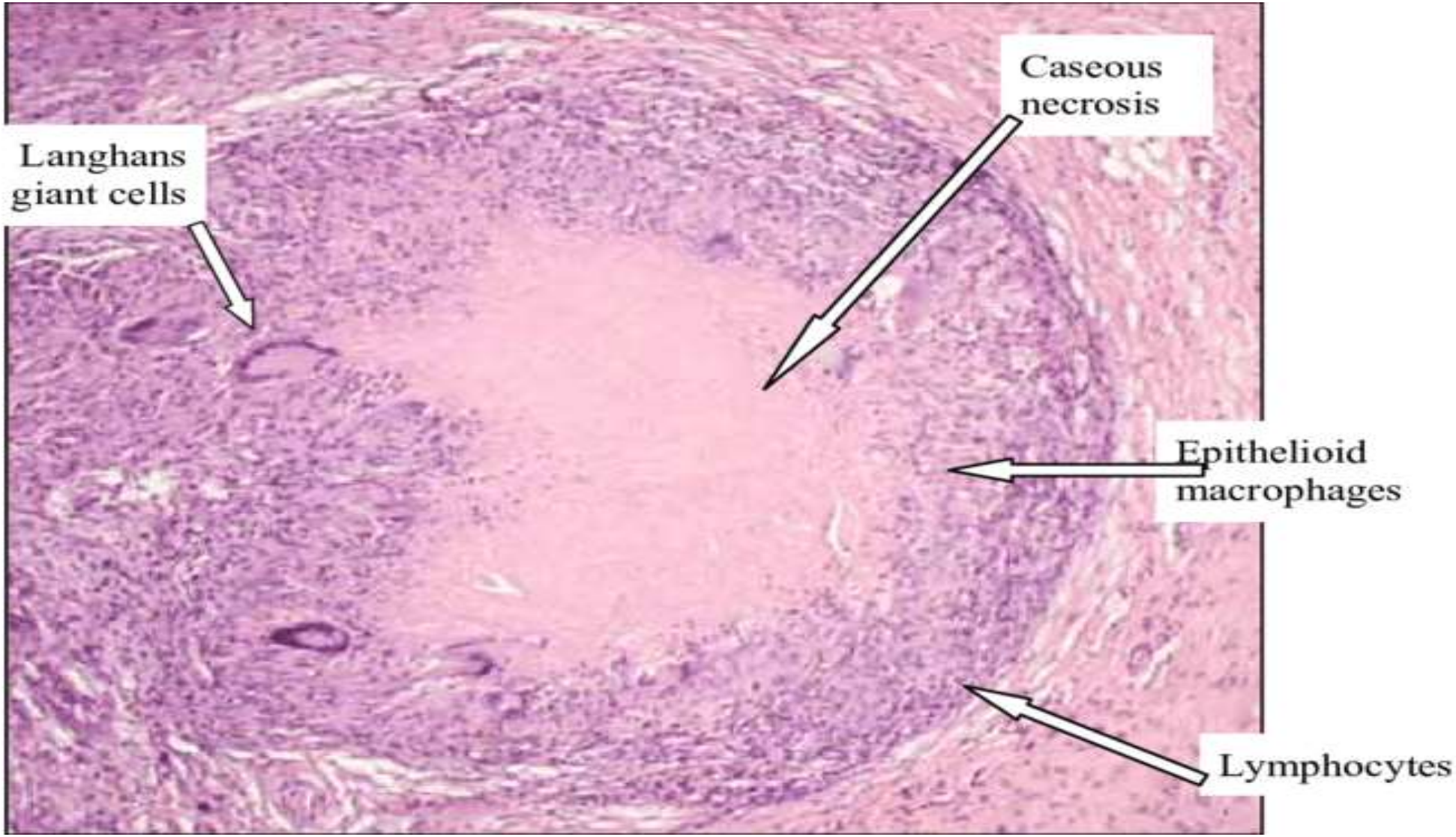
Amorphous granular eosinophilic debritic material
CASEOUS NECROSIS

Modified macrophages with abundant cytoplasm and pale staining "slipper" shaped nuclei
EPITHELOID CELLS

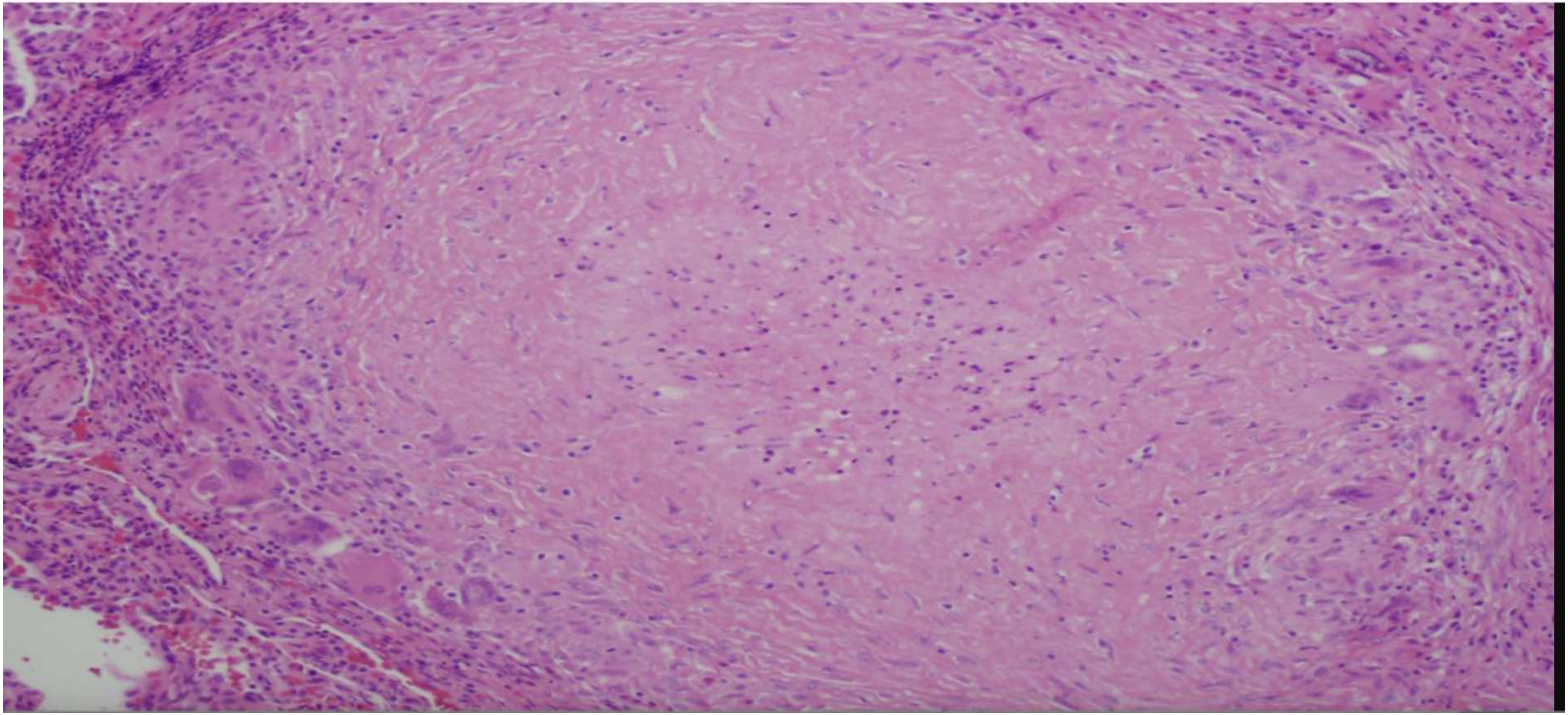
Multinucleated giant cell
LANGHAN GIANT CELL

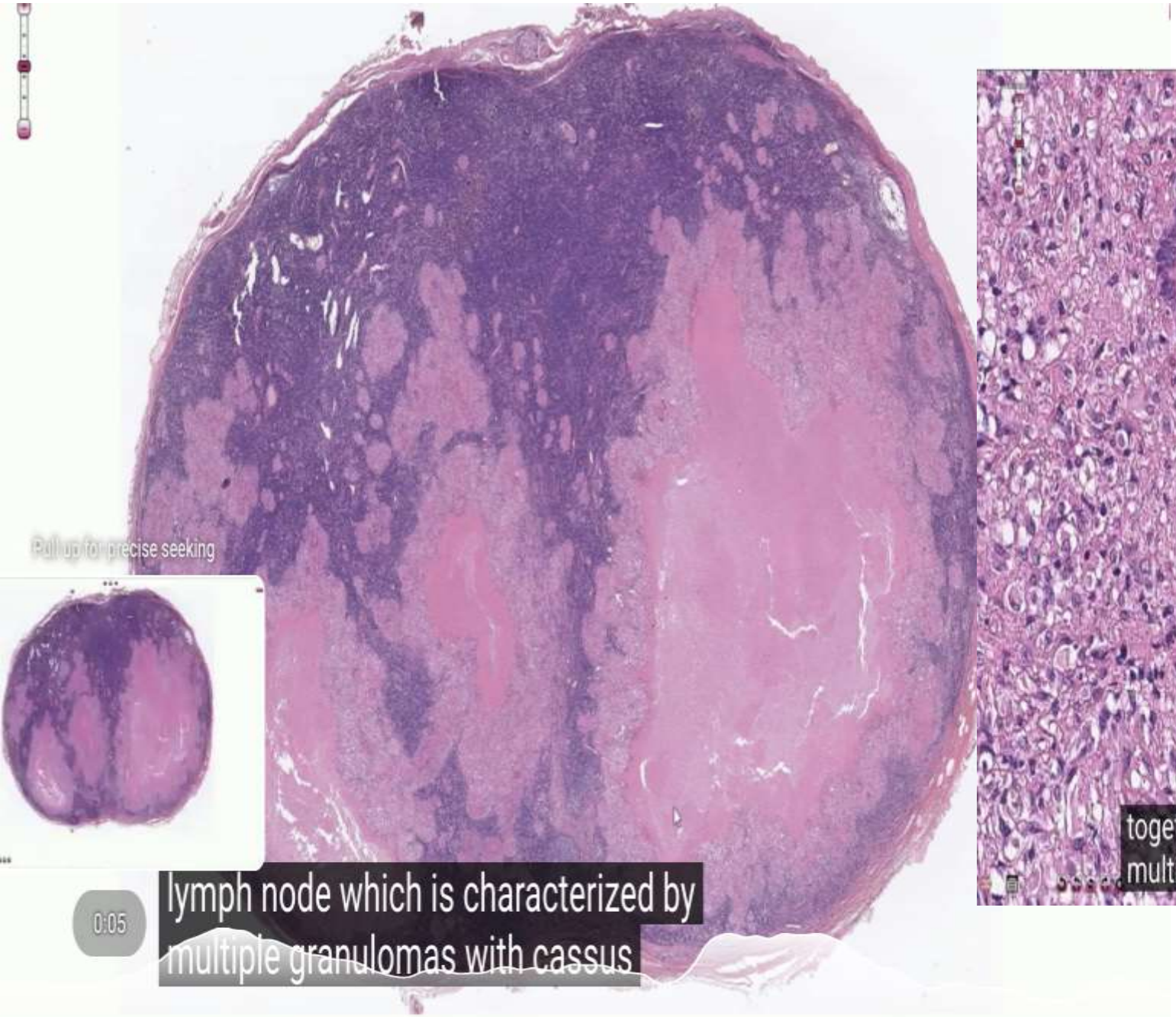
Collar of lymphocytes surrounding epitheloid cell aggregates

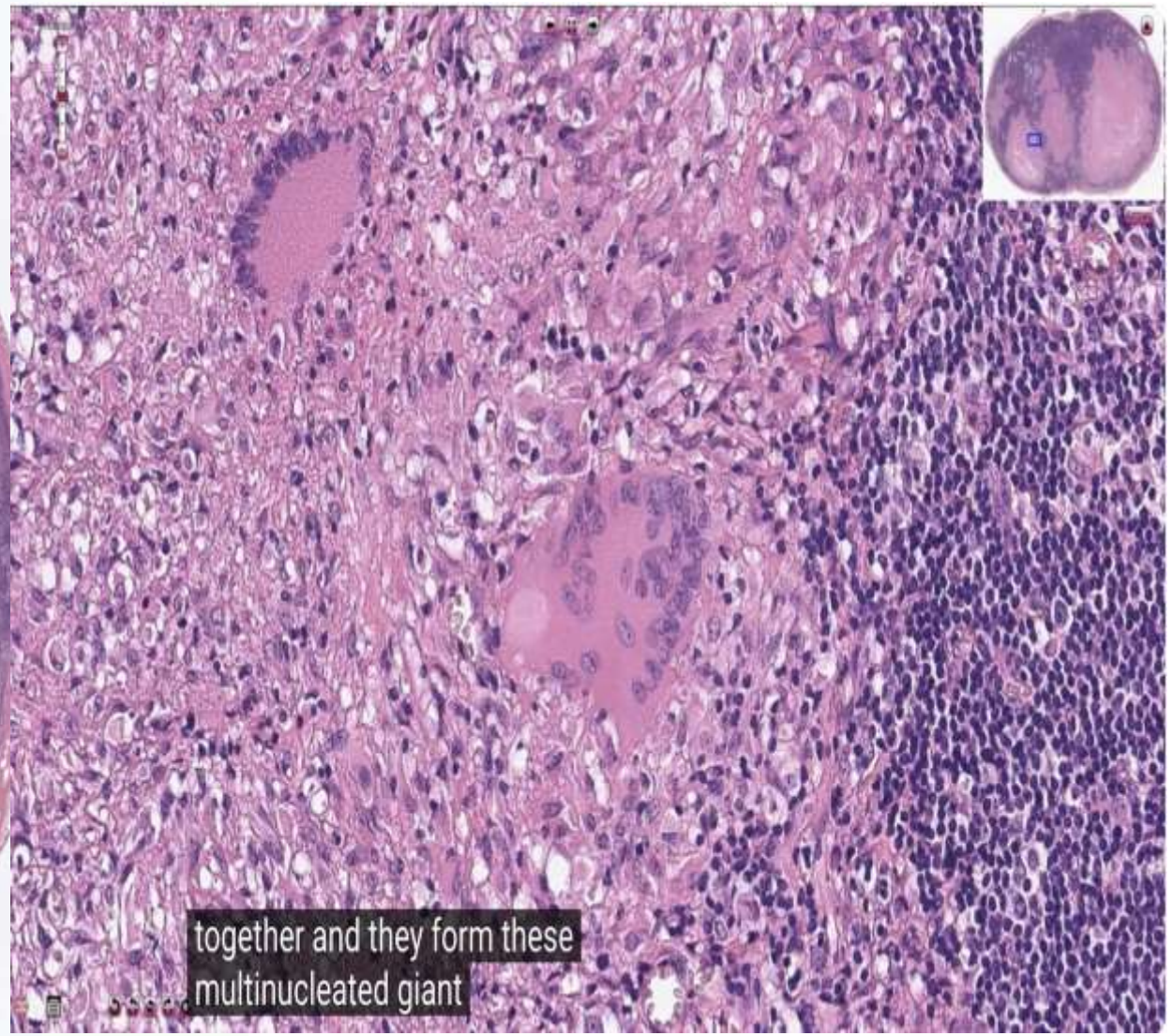
TB granuloma



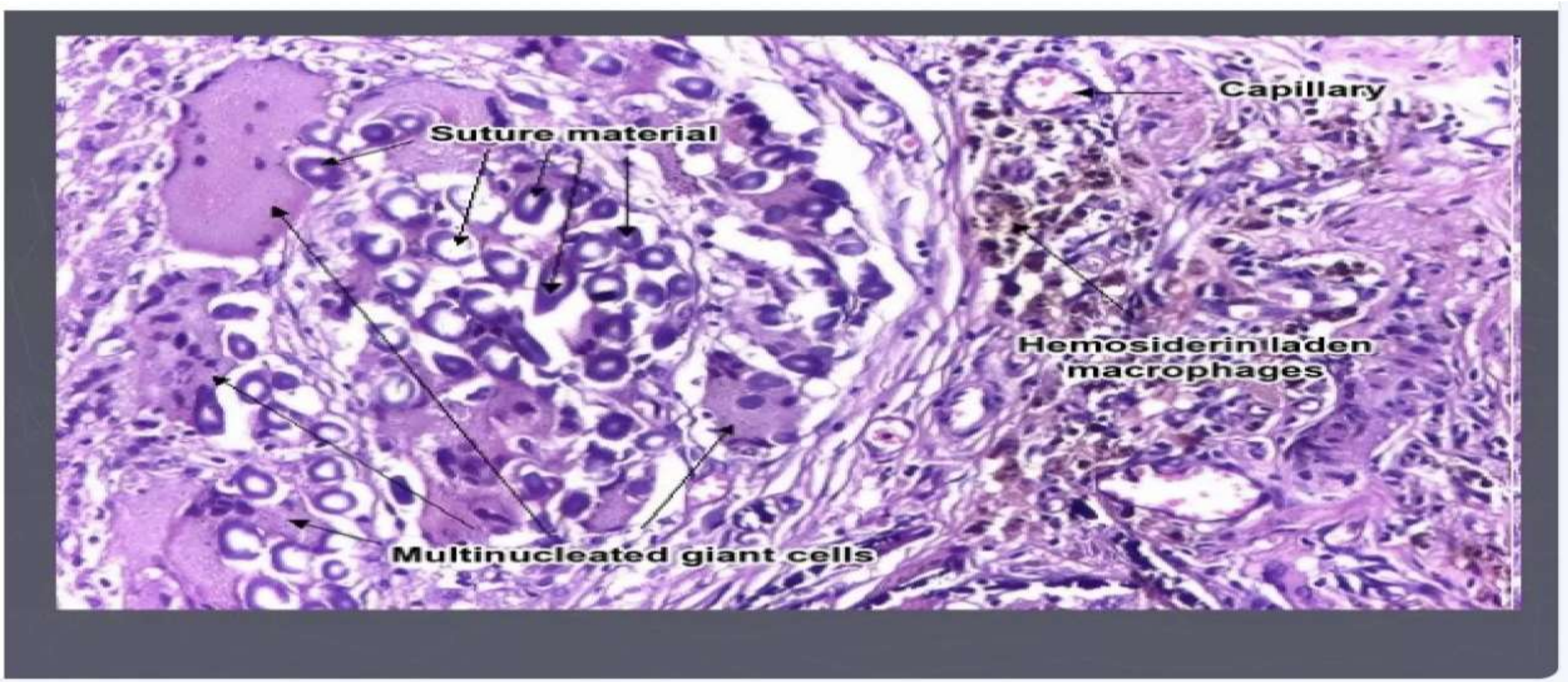
M/F granuloma



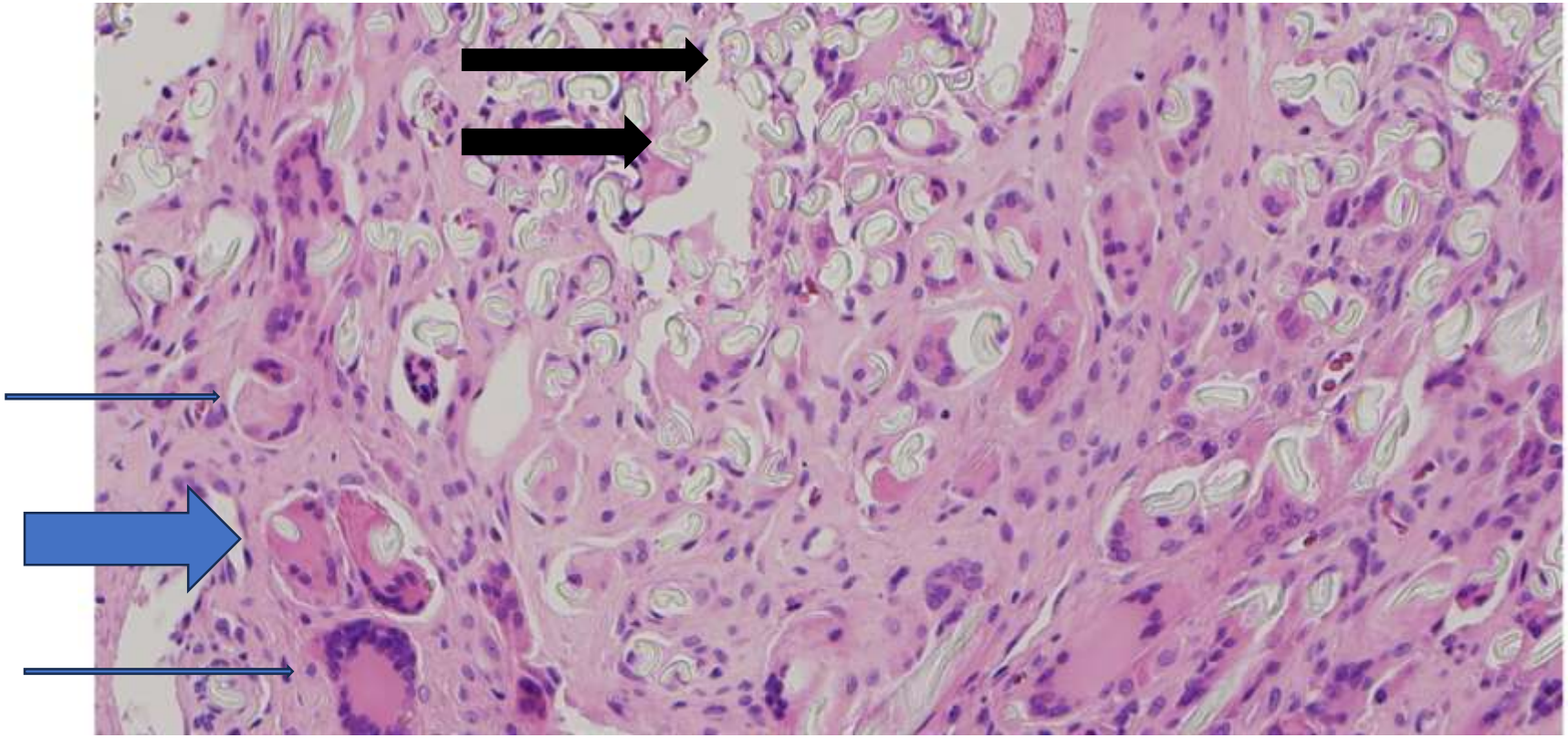

Pull up for precise seeking
lymph node which is characterized by
multiple granulomas with cassus
0:05

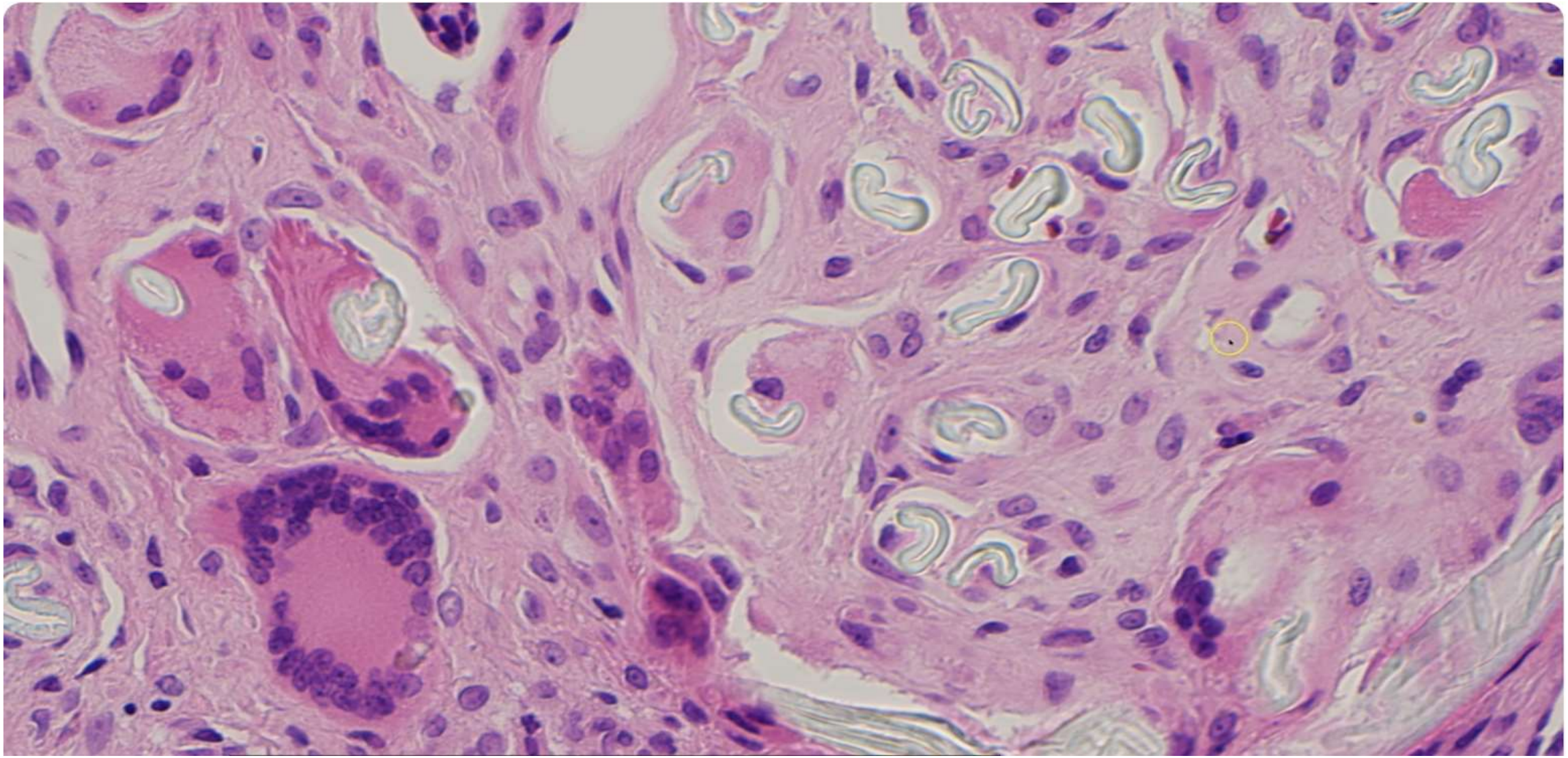

together and they form these
multinucleated giant

Foreign body giant cell



42 years old male with toe amputation



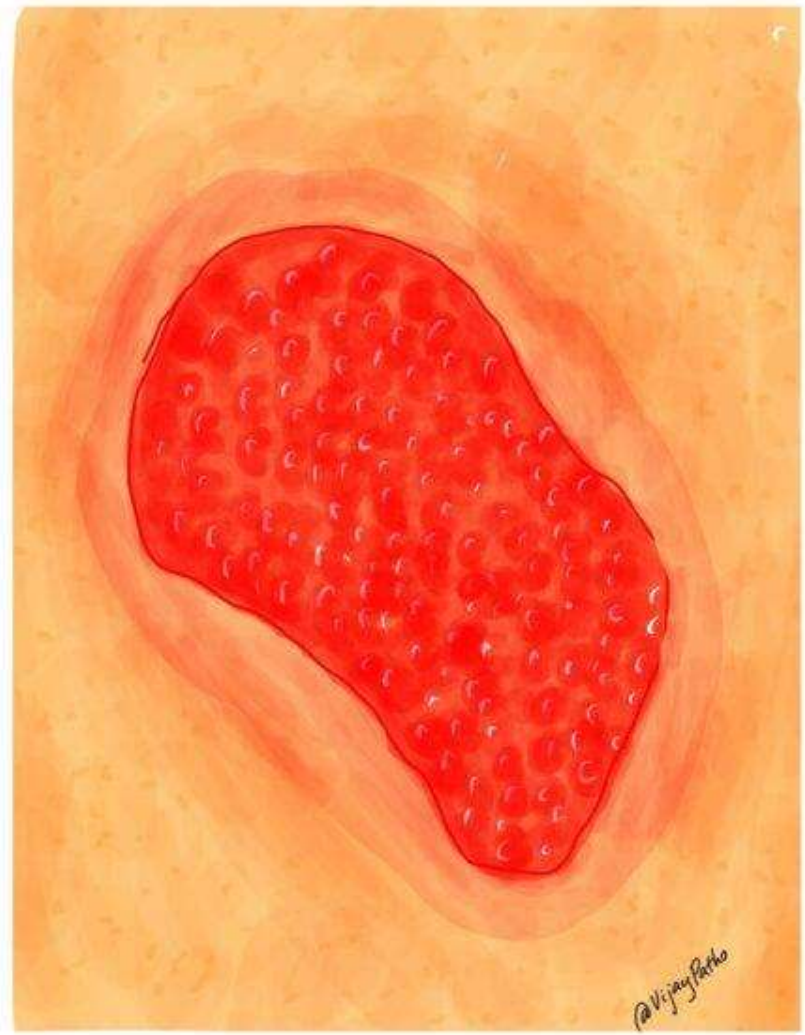


Foreign body giant cells around a gauze material+ its refractile material

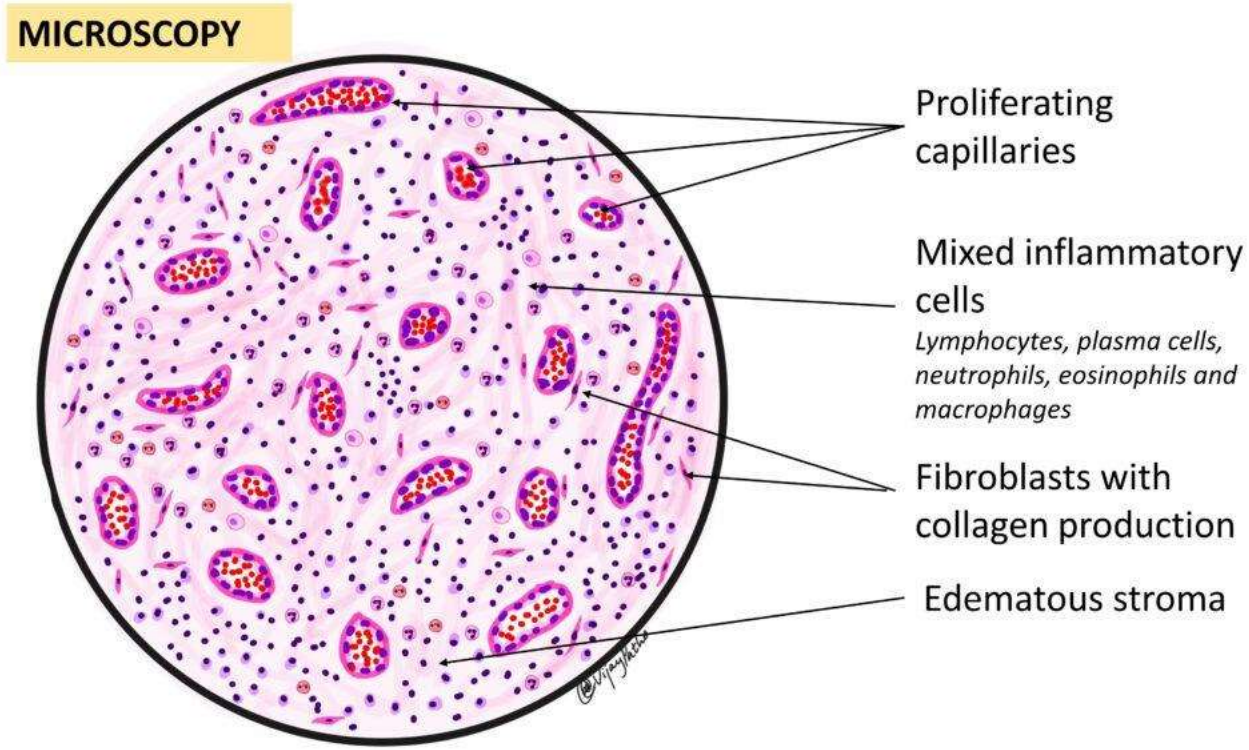
- Healing and repair

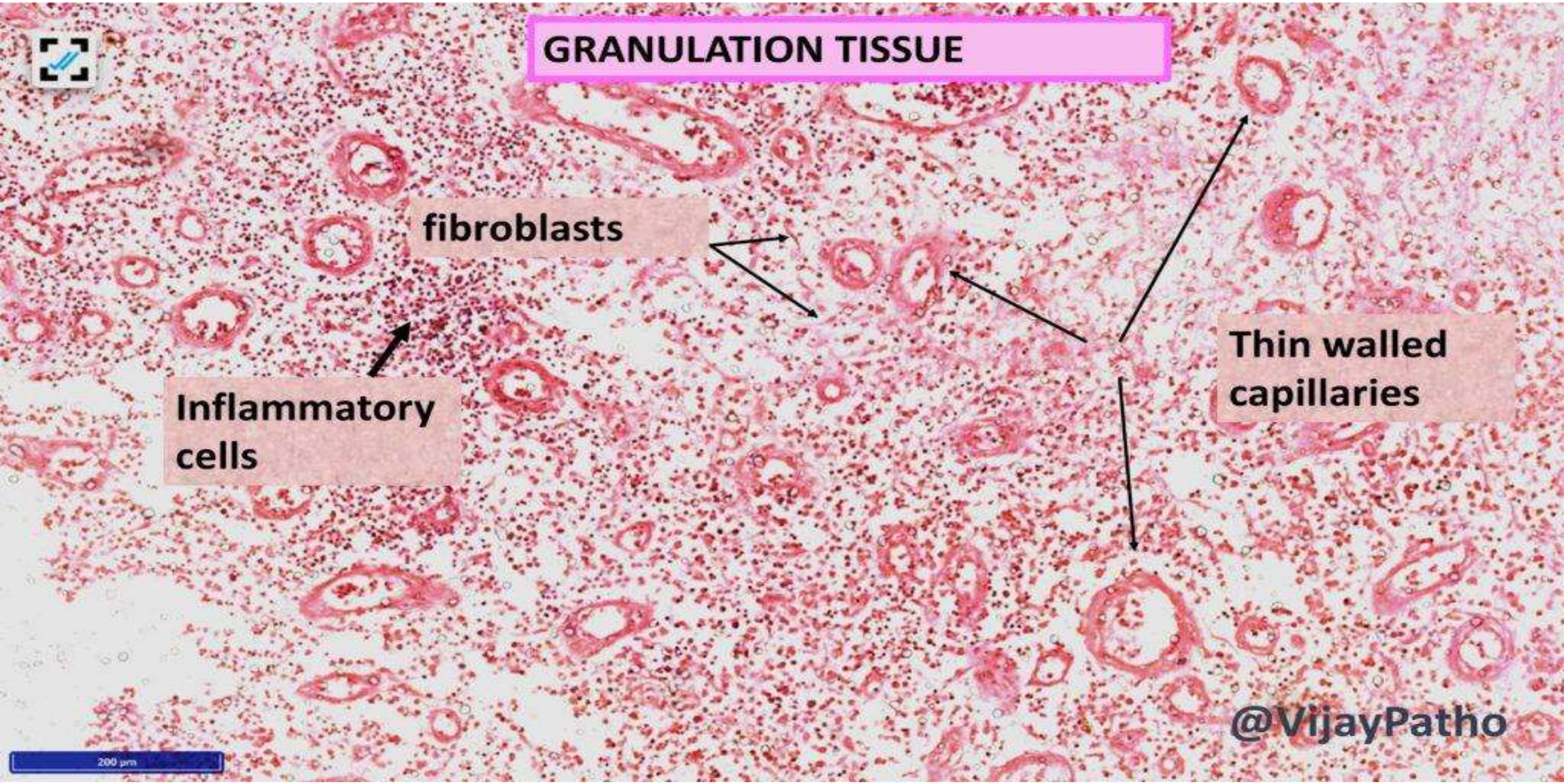
Granulation tissue

GROSS	
Color	Pink to red
Texture	Granular/ small grains
Consistency	Soft to firm
Shape	Fills the wound/ shape of wound
Bleeding tendency	Can bleed easily with minor trauma



Granulation tissue




GRANULATION TISSUE
fibroblasts
Inflammatory cells
Thin walled capillaries
@VijayPatho
200 µm

Wound healing



first intention healing



second intention healing

Complications of wound healing



Wound Contracture



Hypertrophic scar



Keloid formation



Figure 3-32 Keloid. **A,** Excess collagen deposition in the skin forming a raised scar known as *keloid*. **B,** Note the thick connective tissue deposition in the dermis. (**A,** From Murphy GF, Herzberg AJ: Atlas of Dermatopathology. Philadelphia, WB Saunders, 1996, p 219; **B,** Courtesy Z. Argenyi, MD, University of Washington, Seattle, Wash.)

Incisional hernia

